

Cystectomy (removing the bladder)

A cystectomy is an operation to remove the bladder. Your operation will also usually involve removing some areas close to the bladder.

On this page

- [What is a cystectomy?](#)
- [What does the operation involve?](#)
- [Open or laparoscopic \(keyhole\) surgery.](#)
- [Before your operation](#)
- [After cystectomy.](#)
- [Going home](#)
- [About our information](#)
- [How we can help](#)

What is a cystectomy?

A cystectomy is an operation to remove the [bladder](#).

This is one of the main treatments for muscle-invasive or locally advanced [bladder cancer](#). The aim is to cure the cancer. You may have [chemotherapy](#) before or after the operation. These anti-cancer drugs help reduce the risk of cancer coming back.

You may also be offered a cystectomy for non-muscle-invasive bladder cancer if:

- you have a high-risk, non-muscle-invasive bladder cancer
- [BCG treatment](#) is not working
- the cancer keeps coming back even after treatment.

Rarely, a cystectomy is used to help control symptoms of advanced bladder cancer. But it is not often likely to be the most useful treatment in this situation.

What does the operation involve?

Sometimes it is possible to have a partial cystectomy. This is not common but means only part of the bladder is removed.

Usually the surgeon removes the whole [bladder](#). This is called a radical cystectomy. They may also remove nearby areas such as:

- the prostate gland or seminal vesicles – this is where semen is stored
- the womb, ovaries, cervix and part of the vagina
- part, or all, of the urethra
- [lymph nodes](#) linked to the bladder.

If the whole bladder is removed, the surgeon also makes a new way for you to pass urine (pee). This is called a [urinary diversion](#). There are different types of urinary diversion including:

- a [urostomy](#) (bowel or ileal conduit)
- [bladder reconstruction \(neobladder\)](#)
- a [continent urinary diversion](#).

A urinary diversion means you will not pass urine in the same way as before. Before surgery, your surgeon or specialist nurse will explain the operation and the effects this may have on your life. They will talk about urinary diversions and which type may be best for you.

Having your bladder removed is major surgery. With help and support from family members, friends, health professionals and support organisations, people usually manage to cope with life after a cystectomy well. But it is important to understand what the operation involves. This may also include possible effects on your sexual wellbeing and fertility.

Effects on erections and sperm production



Radical cystectomy usually removes the organs that make semen. The operation may also affect the nerves that help with erections.

After surgery, this means you will not be able to make someone pregnant without fertility treatments. If you are worried about this, talk to your doctor or specialist nurse before your surgery. You may be able to [store sperm](#) to use in the future with [fertility treatments](#) to start a pregnancy. But this has to be done before you have treatment or surgery for bladder cancer.

You may also have difficulties getting or keeping an erection. This is called [erectile dysfunction](#) or ED. Your doctor can tell you about treatments that may help improve ED.

Menopause



Surgery that removes the womb and ovaries will cause the menopause if you have not already been through it. This means monthly periods will stop and it is no longer possible to get pregnant.

If you are worried about this, talk to your doctor or specialist nurse before your surgery. Sometimes the surgeon can leave the ovaries in place to prevent early menopause. Your doctor or nurse can also give you information about [managing menopausal symptoms](#) and your options for possible [fertility treatments](#) to start a pregnancy in the future.

Vaginal changes



Removing part of the vagina during surgery makes it shorter. Going through the menopause can also cause vaginal dryness. Both these changes can affect how sex feels after a cystectomy. You may find penetrative sex uncomfortable or painful. Internal examinations can also be more difficult. A vaginal lubricant or moisturiser may help. Or try different sexual positions so you can control the depth of penetration. Sometimes using a vaginal dilator will help. Your cancer doctor or nurse can give advice about this. We have more information about [female pelvic side effects and your sex life](#).

Open or laparoscopic (keyhole) surgery

A cystectomy and urinary diversion may be done as an open operation or as keyhole surgery. Keyhole surgery is also called laparoscopic surgery. Your surgeon will talk to you about which type of surgery is likely to be best in your situation. Some types of surgery will not be available in all hospitals. You may need to be referred to another hospital to have it done.

An open cystectomy means the surgeon makes one cut in your lower tummy (abdomen) to do the operation.

Laparoscopic surgery means the surgeon makes several small cuts in the tummy. They pass a thin tube with a light and camera on the end called a laparoscope into the tummy through one of the cuts. They use surgical tools through the other cuts to remove the cancer.

Keyhole surgery may have less complications than open surgery, and wounds may heal faster. This may mean you recover more quickly.

Sometimes the surgeon uses a machine (robot) during keyhole surgery. This is called robotic-assisted surgery. The surgeon controls a robotic tool that holds the instruments. Your surgeon can explain more about the benefits and disadvantages of this type of surgery.

Before your operation

Before your operation, you go to a pre-assessment clinic for tests to check you are well enough to cope with the operation. These can include:

- [blood tests](#)
- a blood pressure check
- a recording of your heart called an [ECG](#).

Some people will have further tests.

You will meet a member of the surgical team to discuss the operation. This is a good time to ask questions or talk about any concerns you may have about the operation. If you think you will need help when you go home, tell them as soon as possible. They can help you with this.

Some hospitals follow an enhanced recovery programme. This aims to reduce the time you spend in hospital and speed up your recovery. It also involves you more in your own care. For example, you will be given information about diet and exercise before surgery. You may be given supplement drinks to take too.

Before the operation, you may have:

Antibiotics



You may have antibiotics to prevent infection. You may have them as an injection or as tablets.

An enema



Because part of your bowel is used for a urinary diversion, you may need an empty bowel for the surgery. You may have fluid given into the back passage (rectum) to help empty the bowel. This is called an enema. It is usually given the evening before the operation.

You usually go to the hospital on the morning of the operation. You will be given elastic stockings to wear during the operation and for some time afterwards. These are called TED stockings. They help prevent blood clots in your legs.

Planning your urinary diversion

For a [urostomy](#), the nurse or doctor carefully plans the position of the opening. This opening is called a stoma. They make a mark on the tummy, so the surgeon knows the best place for the stoma. They do this even if you are planning to have a [bladder reconstruction](#). This is in case they need to make a urostomy.

A stoma is often placed on the right side of the belly button (navel). But if you are left-handed, it can be positioned on your left side. It is usually placed to suit your needs.

If you are having a [continent urinary diversion](#), the stoma is placed where you can see it and put a catheter into it.

After cystectomy

After the operation, you will be looked after in a high-dependency or intensive care unit for 1 or 2 days.

The nurses will encourage you to start moving about as soon as possible after your operation. This helps to prevent problems such as chest infections or blood clots. They will encourage you to do regular leg movements and deep breathing exercises. A physiotherapist or nurse can explain these to you.

You may have loose or runny stools (poo) after your surgery. This is because part of the bowel is used to make a urinary diversion. It usually gets better over time. Tell your doctor or specialist nurse if it continues.

Drips and drains

At first, you have fluids through a drip (infusion) into a vein in your hand or arm. You may be given painkillers through your drip. Once you are eating and drinking again, the drip is removed.

Some people may have a tube that goes up the nose and down into the stomach. This is called a nasogastric tube. It is used to remove fluid from the stomach, so you do not feel sick. This is usually removed 1 or 2 days after surgery.

You may have a tube close to the operation wound to drain fluid away. A nurse will take this out after a few days, when fluid stops draining.

You will have tubes to help drain urine (pee) from your body. The tubes you have depend on the type of urinary diversion you have. Your doctor and nurse will give you more information.

Pain

After your operation, you will need medicines to manage any pain for a few days.

To begin with, you may be given painkillers into a vein using an electronic pump. You control the pump using a hand control that you can press when you need more of the painkiller. This is called patient-controlled analgesia (PCA). It is fine to press the hand control whenever you have pain. The nurse sets the pump so you cannot have too much painkiller.

Some people are given painkillers into their back. This is called an epidural. The drugs numb the nerves in the area where you had the operation to control your pain. You usually have them through a drip (infusion) attached to an electronic pump.

Sometimes during the operation, the surgeon puts small tubes into the area around the wound. After the operation, your doctor or nurse uses these tubes to give you painkillers. This helps to numb the nerves in the same way as an epidural.

If you are in pain, tell your nurse or doctor straight away. You will be given painkillers to take before you go home.

The wound

The surgeon uses stitches or staples to close the wounds during surgery. They will explain when these need to come out and will arrange for a nurse to do this. Wound infections can be a complication of the surgery. Signs the wound is infected include:

- heat
- redness
- swelling
- fluid or pus coming from the wound
- feeling unwell
- a fever or high temperature.

Tell your nurse or doctor if you have any of these symptoms, even after you go home. If you are unable to talk to your hospital team, tell your GP or out-of-hours service.

Going home

How long you are in hospital for will depend on the operation you have had and how quickly you recover. It can be anything from 5 to 14 days.

Your surgeon will tell you how soon you can get back to doing things such as work, driving, shopping, gardening or playing sport. This will depend on the operation you had and your recovery.

Before you leave hospital, you will be given an outpatient appointment for a follow-up visit a few weeks after your surgery. It is a good time to discuss any concerns you may have after your operation. Your doctor will tell you if you need any further treatment.

About our information

References



Below is a sample of the sources used in our bladder cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

Mottet N, Bellmunt J, Briers E, et al. Non-muscle-invasive bladder cancer (TaT1 and CIS). European Association of Urology (Internet), 2021. Available from uroweb.org/guideline/non-muscle-invasive-bladder-cancer (accessed September 2021).

Witjes JA, Bruins HM, Cathomas R, et al. Muscle-invasive and metastatic bladder cancer. European Association of Urology (Internet), 2021, Available from uroweb.org/guideline/bladder-cancer-muscle-invasive-and-metastatic (accessed September 2021).

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, [Dr Ursula McGovern](#), Consultant Medical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

Date reviewed

Reviewed: 01 November 2022 | Next review: 01 November 2025

ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help